

Tenet Healthcare Corp **THC**

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by

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2026 2027

Price:	194.38	EPS	17.88	17.68
Shares Out. (in M):	86	P/E	10.9	11.0
Market Cap (in \$M):	16,743	P/FCF	7.8	10.6
Net Debt (in \$M):	10,288	EBIT	2,740	2,683
TEV (in \$M):	27,031	TEV/EBIT	9.9	10.1
Borrow Cost:	General Collateral			

Description

\$194.38 | Target \$110-149 | 12-24 months

Note: EBIT and FCF are net of the partners' non-controlling interest (\$930M in 2026, \$1,012M in 2027).

At \$194.38 Tenet is priced for growth and profitability while earnings are becoming increasingly flat. Tenet runs hospitals and surgery centers, but three things are draining the profit that reaches shareholders: physician partners take a growing cut of its best business, anesthesia costs are exploding, and pricing is slowing. **My base case is \$149 (25% downside) and my bear case \$110 (45% downside).** Net debt is \$10.3B, at 2.2x EBITDA. Insiders sold \$38.9M of stock over the past year with no buyers, among them the CEO, COO, CIO, general counsel and a director. Borrow is easy at 4.3% of the float with 2.5 days to cover.

THC is a short because:

- **Anesthesia subsidies have tripled to \$221M** and reach roughly \$580M by 2030, 12.5% of company profit, as a national anesthesiologist shortage forces Tenet to pay up to keep its operating rooms staffed.
- USPI, Tenet's surgery-center arm, has **grown by buying centers, and that buying has collapsed** from 16.7 points of revenue growth in 2021 to a guided 3.8 this year, while pricing at the centers it already owns has slowed from 9.1% to 5.6%.
- **Physicians co-own USPI's centers and take a share of the profit** before shareholders do, and Tenet guides that share up 15.6% to 20% of company profit while guiding EBITDA growth flat.
- **Tenet underinvests in its hospitals**, spending about half of depreciation on a fleet that averages 1.91 of 5 stars, and demand is now falling as the ACA losses HCA flagged on July 14 hit its 70%-commercial book harder than peers.

Business Model and Background

Tenet runs two segments, and 76% of revenue sits in the low-margin, Hospital segment. Hospital Operations is 47 acute-care hospitals (full-service, with emergency rooms and overnight stays) in Texas, California, Florida, Arizona and Michigan, at \$16.1B on a 16% margin. It houses Conifer, a billing unit that loses its largest outside client, CommonSpirit, on **December 31 2026 (about \$630M of recurring revenue)**. The profit engine is the smaller half, Ambulatory Care, run by USPI (United Surgical Partners International): \$5.2B of revenue at a 39.2% margin across 559 outpatient surgery centers (same-day procedures, no overnight stay) in 37 states.

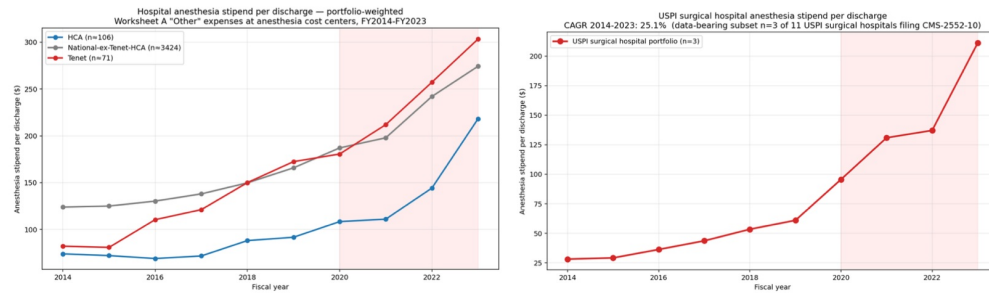
Each USPI center is a joint venture, and its surgeon-owners take 20-40% of the profit before Tenet's shareholders receives them, a share called non-controlling interest (NCI) that reached 39.7% of the segment's EBITDA in 2025. USPI has also bought much of its growth, more than \$3.5B of acquisitions since 2019, in a fragmented market of 10,200 centers where no operator tops 8% and the surgeons decide where cases go based on their profitability. It's one integrated rival, **UnitedHealth's Optum, both owns surgery centers and insures 29.7 million commercial members** it can steer to them.

#1 Exploding Anesthesia Cost

Tenet's anesthesia subsidy has tripled to \$221M since 2014 and, on its current path, reaches 12.5% of company profit by 2030. The subsidy exists because anesthesia groups are separate businesses that staff Tenet's operating rooms and bill insurers on their own. For decades they funded themselves by staying out of network and billing patients at 3-5x Medicare rates, which produced 20-40% of their revenue. The No Surprises Act, which took effect in January 2022, banned those surprise bills and replaced them with arbitration benchmarked to the median in-network rate, capped near twice Medicare. Industry anesthesia revenue fell 20-35%, and staffing a hospital stopped covering the group's costs.

Envision Healthcare, the physician-staffing firm KKR took private, filed for Bankruptcy in May 2023, citing \$7.7B of debt and the "flawed" implementation of the law. Groups that stayed solvent turned to their facilities for help, demanding cash subsidies called stipends. A facility that will not pay loses its anesthesia coverage and cannot run its operating rooms, so the subsidy became mandatory. US hospitals' stipend bill went from \$3.69B in 2014 to \$8.26B in 2023, rising 45% in the two years after the law alone, from \$5.70B in 2021.

At Tenet's hospitals the subsidy per patient nearly quadrupled, from \$82 in 2014 to \$303 in 2023, compounding **15.6% a year, faster than its competitor HCA (12.7%) or the national average (9.2%)**.



Source: CMS HCRIS, Form CMS-2552-10 Worksheet A anesthesia cost-center "Other" expenses per discharge, FY2014-2023.

Texas, Florida and Arizona hold over 40% of USPI's revenue, and Tenet's hospitals there are compounding stipends fastest, at 24.6% a year.

Of the eleven USPI surgery centers filing public cost reports, the **three with complete data compounded anesthesia costs 25.1% a year (2014-2023) and 31.7% since 2018**, the same inflation trend the hospitals show. Baylor Scott and White Surgicare Waxahachie ran 26.6% over nine years. VMG Health's 2025 survey found the share of surgery centers paying stipends jumped from 28% to 44% in one year, and 67% ranked anesthesia a top three financial challenge for 2026.

The American Society of Anesthesiologists projects a shortfall of 8,000-12,000 anesthesiologists by the mid-2030s, 20-30% of the workforce, with 59% of those practicing now aged 55 or older. Residency slots are capped by Congress and training a replacement takes about a decade, so the shortage driving these subsidies cannot ease within the forecast.

The subsidy doubles as a share of profit, from 6.0% of EBITDA in 2023 to 12.5% by 2030 adding \$361M of annual cost as the bill climbs from \$221M to \$582M. The ambulatory piece grows 5x over that span against 2.3x for the hospitals, so the fastest inflation lands on USPI, the 39%-margin segment the stock is priced for.

Fiscal year	2014	2019	2023	2026E	2027E	2028E	2029E	2030E
Hospital (\$M)	66	125	197	283	320	361	408	461
USPI ambulatory (\$M)	3	10	24	48	60	76	96	121
Combined (\$M)	69	135	221	331	380	437	504	582
Combined, % of consolidated EBITDA	2.9	5.3	6.0	7.1	8.2	9.4	10.9	12.5

Source: CMS HCRIS hospital actuals through 2023 projected at 12.9%, USPI estimated at 0.34% of revenue (its surgical hospitals, 2023) grown at 26%, EBITDA held at mid-point of FY2026 guidance.

#2 The higher-margin mix shift that paid the anesthesia bill is exhausting

Same-facility surgical cases, the volume at centers USPI already owns, grew just 0.3% in FY2024 and 0.3% again in FY2025, and ranged from -2.1% to +2.1% across the nine quarters to Q1 2026. USPI is **not growing by doing more operations**, but rather from **charging more per case**, and Tenet has guided that revenue growth down to 3.0-6.0% for FY2026 against 7.5% in FY2025.

Quarter	Same-facility revenue per case growth (%)	Same-facility system-wide revenue growth (%)	Same-facility surgical case growth (%)
2024 Q1	6.8	6.4	-0.4
2024 Q2	6.8	7.1	0.2
2024 Q3	7.6	8.7	1.0
2024 Q4	8.5	8.6	0.1
2025 Q1	9.1	6.8	-2.1
2025 Q2	8.3	7.7	-0.6
2025 Q3	6.1	8.3	2.1
2025 Q4	5.5	7.2	1.6
2026 Q1	5.6	5.3	-0.3

Source: Tenet quarterly earnings releases, Q1 2024 through Q1 2026

Medicare's surgery-center rates rose a median 3.8% a year from 2018 to 2026 across a constant 3,384-procedure panel, but that rate is only one of two levers behind per-case revenue. The bigger one is mix, shifting toward joint replacement. Medicare added total knee to the surgery-center list in 2020 and total hip in 2021, paying \$9,393 and \$9,614 against a \$1,645 median procedure, so moving one case from an average procedure to a knee **multiplies its revenue 5.7x with no extra surgery**.

USPI's segment margin, excluding grant income, **fell from 41.7% in FY2021 to 40.7% in FY2022, 39.9% in FY2023 and 39.2% in FY2025**, even as per-case revenue grew 6-9% a year. Therefore, this mix-shift towards higher cost operations has not been able to sufficiently cover up the rising anesthesia bill.

Reimbursement rates for two of the highest cost operates, Total knee has risen just 1.4% a year and total hip 1.7% since 2021, against a 3.8% median update and 3.9-4.8% for the cataract, colonoscopy and arthroscopy cases. CMS is holding down the exact joint rates USPI leaned on, and it **cut the knee rate 2.9% in 2024**.

CMS ASC national payment rate	2021	2026	Change	Per year
Total knee replacement (27447)	\$8,759	\$9,393	+7.2%	1.4%
Total hip replacement (27130)	\$8,818	\$9,614	+9.0%	1.7%
Shoulder arthroscopy, rotator cuff (29827)	\$2,929	\$3,696	+26.2%	4.8%
Colonoscopy with biopsy (45380)	\$524	\$657	+25.3%	4.6%
Knee arthroscopy, meniscectomy (29881)	\$1,328	\$1,645	+23.8%	4.4%
Cataract removal with lens (66984)	\$1,039	\$1,256	+20.8%	3.9%
Spinal injection, lumbar (64483)	\$416	\$486	+16.8%	3.2%

Source: CMS Addendum AA, national rates.

#3 The partners take a rising share, and the surgeons are leaving

The partners' cash take has grown 17.6% a year since FY2021, far faster than the 10.9% growth in its EBITDA. These partners are the physicians and health systems that co-own USPI's centers, and their share of consolidated EBITDA, the non-controlling interest, has climbed from **13.8% to 17.4% in four years**.

Period	Consolidated Adj EBITDA (\$M)	Distributions paid to NCI (\$M)	Distributions as % of EBITDA	Purchases of NCI (\$M)
FY2021	3,073	423	13.8%	27
FY2022	3,128	560	17.9%	100
FY2023	3,682	594	16.1%	167
FY2024	4,044	681	16.8%	200
FY2025	4,643	809	17.4%	92
FY2026 guide	4,485-4,785	900-970	20.2% (mid)	not guided
Q1 2025	1,163	189	16.3%	41
Q1 2026	1,162	197	17.0%	549

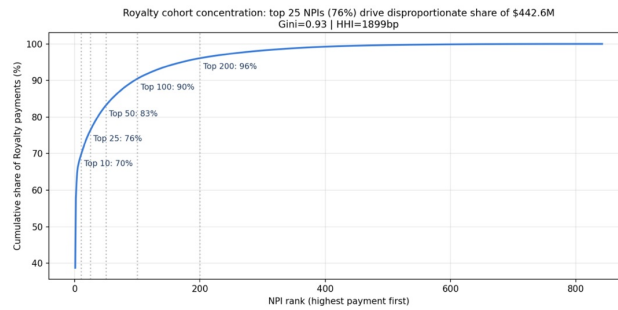
Sources: Tenet cash flow statements and earnings releases. The \$549M Q1 2026 purchase is the CommonSpirit Conifer redemption, not a USPI partner buyout.

Tenet guides no profit growth for 2026 but a 15.6% raise for its partners. Consolidated EBITDA is guided flat at the midpoint, and Q1 already printed flat at \$1,162M against \$1,163M. The partner distribution is guided up 15.6%, lifting their share to **20.2% of company EBITDA**, and Q1 distributions rose only 4.2%, so most of that raise is still ahead.

Tenet spent **\$92M buying back partner stakes in FY2025, against the \$809M it paid them that year**, so the buyouts barely dent the payout.

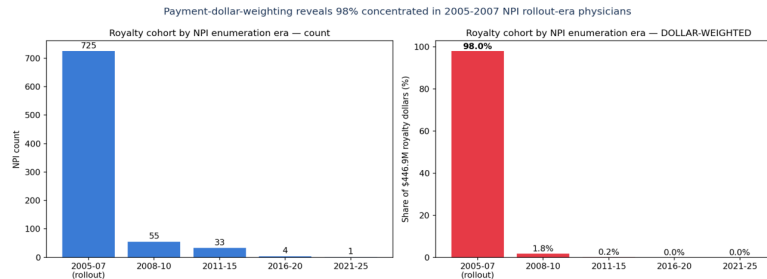
Allison v. Southwest Orthopaedic Specialists, a 2018 complaint in the Western District of Oklahoma, quotes USPI's operating agreement: "The other physician owners of OCOM Equity have **buy-out provisions at a 4.0 EBITDA multiple**." The same filing describes **a 6.5x multiple granted to selected high referrers** as "a 62.5% premium over fair market value and what other physicians would receive," which prices fair market value at 4.0x. Tenet retires the equity at that fair price multiple, but it cannot retire the referral pattern, which follows the surgeon to whichever center recruits a successor.

A surgeon on r/whitecoatinvestor described the worth of stake re-selling at buyout: **"Initial investment was \$9,000 per 1% ownership"**... We recently closed a deal to sell a majority of our stake in the ASC to a publicly traded company for over **\$275,000 per 1% ownership**."



Source: CMS Open Payments royalty and license payments to the 842 surgeons, by NPI, ranked highest first.

Device-maker royalty payments, a public proxy for which surgeons drive the most business, total \$446.9M across 842 USPI-affiliated surgeons, and the top 10 capture 70% of them (CMS Open Payments). The top 50 capture 83% and anchor just 39 of USPI's 559 facilities, so two or three retirements at one of them is a concentrated claim on both cash and case volume.



Source: CMS Open Payments royalty and license payments by NPI, grouped by NPPES enumeration year.

Of the 842 surgeons, the 725 who registered during the **2005-2007 rollout of national provider IDs hold 98% of the royalty value (\$428.4M of \$437M identifiable)**, which puts them at 50-75 age today (CMS Open Payments, NPPES). Surgeons who registered after 2010 hold under 0.2%, showing that the people directing USPI's most valuable volume are nearing retirement.

Mandatory Buyout Obligation Sensitivity (\$M): 2005-2007 NPI Cohort						
EBITDA Multiple \ % Retiring	30%	50%	70%	90%	100%	
4.0x	\$ 240	\$ 400	\$ 560	\$ 720	\$ 800	
4.5x	\$ 270	\$ 450	\$ 630	\$ 810	\$ 900	
5.0x	\$ 300	\$ 500	\$ 700	\$ 900	\$ 1,000	
5.5x	\$ 330	\$ 550	\$ 770	\$ 990	\$ 1,100	
6.0x	\$ 360	\$ 600	\$ 840	\$ 1,080	\$ 1,200	
6.5x	\$ 390	\$ 650	\$ 910	\$ 1,170	\$ 1,300	

At the contractual 4.0x multiple, a retirement wave among these surgeons **costs Tenet \$400M if half go and \$560M if 70% do, against the \$92M a year it now spends on buyouts**. That is four to six years of buyout budget landing in a compressed window, a mandatory cash claim that competes with the buyback. Any premium Tenet must pay to retain them pushes it higher: **5.0x takes the 70% case to \$700M**.

The partners' share climbs from 17.4% toward a guided 20.2% of EBITDA while the surgeons who supply the cases age out with no successors, so USPI's terminal growth leaks to the labor force rather than shareholders.

#4 The hospital half, under-maintained and losing exchange patients

Hospital Operations is 76% of revenue and 56% of profit, and consensus holds its 16% margin flat through FY2028. Tenet holds that margin by under-investing in the hospitals, and since HCA's July 14 warning the patient demand behind them is falling too.

Tenet's 47 hospitals average 1.91 of 5 on Medicare's Star Ratings, a quality score that determines hospital payment, with none rated 4 or 5. 16.2% of its hospitals are penalised under all three of Medicare's quality programs at once, and 71% sit in the bottom quartile of the largest, Value-Based Purchasing.

Buyers of former Tenet hospitals immediately spend more than double to fix them, \$43-45K per bed a year against Tenet's \$17-19K, and 12 of 17 divested hospitals show higher capex under new owners. Ten sold in 2015-2018 now score 67.1% on patient surveys against 56.8% at hospitals Tenet kept. Its own capex ran just 0.55x depreciation from 2015 to 2022 against HCA's 1.10x, less than half what it takes to hold the asset base flat.

Pandemic-era subsidies that made ACA exchange insurance cheap expired at the end of 2025, so 2026 premiums jumped and enrollees are dropping coverage, showing up uninsured or not at all. Tenet guided a \$250M FY2026 hit on its Q4 call, the low end of a \$223-352M range across analysts. It draws 70.2% of revenue from commercial and managed-care patients against HCA's 52%, making the coverage loss reaches more of its base.

On July 14 HCA cut its own FY2026 guidance on a \$400M quarterly hit from patients "who lost coverage on the health insurance exchanges," with **enrollment down 13%, worse than it had modelled**, and Tenet has not yet pre-announced. A further blow lands in 2027-28: Medicaid supplemental payments, extra state funds hospitals rely on, face cuts of \$1,338M, 8% of Tenet's hospital revenue.

Consensus holds that 16% margin flat through 2028, but it has to absorb the \$250M exchange loss now, the \$1,338M

Medicaid cut in 2027-28, and years of deferred maintenance, all at once.

Valuation

Tenet trades at 7.33x Adjusted EBITDA less NCI, roughly 15% below the 8.93x peer median. With the current multiple at 7.33x, the only variable is FY2028 EBITDA less NCI, and at \$199.20 the price embeds about \$3,744M, this year's shareholder EBITDA held flat to 2028.

Comps: EV excl. NCI	Price (\$)	EV excl. NCI	FY1 EBITDA	NCI (FY25A)	EBITDA - NCI	EV / (EBITDA-NCI)
Tenet Healthcare (THC)	\$199.20	\$ 27,446	\$ 4,674	\$ 960	\$ 3,714	7.39x
HCA Healthcare (HCA)	\$371.18	\$ 127,794	\$ 15,858	\$ 998	\$ 14,860	8.60x
Universal Health (UHS)	\$151.16	\$ 13,910	\$ 2,696	\$ 22	\$ 2,674	5.20x
Encompass Health (EHC)	\$112.49	\$ 13,535	\$ 1,370	\$ 193	\$ 1,177	11.50x
Community Health (CYH)	\$3.38	\$ 10,612	\$ 1,354	\$ 167	\$ 1,187	8.94x
Peer median (excl. THC)						8.77x

Unlevered Free Cash Flow	FY2023	FY2024	FY2025	FY2026E	FY2027E	FY2028E	FY2029E	FY2030E
NOPAT	\$ 2,023	\$ 4,602	\$ 2,966	\$ 2,607	\$ 2,370	\$ 2,348	\$ 2,158	\$ 2,029
+ Depreciation & Amortization	\$ 870	\$ 818	\$ 863	\$ 840	\$ 865	\$ 886	\$ 908	\$ 930
+/- Change in Net Working Capital	\$ -	\$ (49)	\$ (87)	\$ (15)	\$ 24	\$ (36)	\$ (10)	\$ (19)
- Capital Expenditures	\$ (751)	\$ (931)	\$ (750)	\$ (1,030)	\$ (1,026)	\$ (1,050)	\$ (1,074)	\$ (1,095)
Unlevered Free Cash Flow	\$ 2,142	\$ 4,440	\$ 2,992	\$ 2,402	\$ 2,233	\$ 2,148	\$ 1,982	\$ 1,846

My base case holds **\$3,149M, 16% below that flat level, worth \$149, a 25% downside**. The bear holds **\$2,699M, worth \$110, a 45% downside**. Even consensus of \$3,871M implies only \$210, 5% above spot, so the price already pays for flat-to-modest growth while the four thesis points pressure the returns down.

	Base Case (60%)	Bull / Consensus (20%)	Bear Case (20%)
FY2028E Adj EBITDA less NCI (\$M)	3,149	3,871	2,699
Exit Multiple (EV / EBITDA, x)	7.33x	7.33x	7.33x
Enterprise Value (excl. NCI)	23,084	28,382	19,785
(-) Net Debt	(10,288)	(10,288)	(10,288)
Implied Equity Value	12,796	18,094	9,497
÷ Shares Outstanding (M)	86.1	86.1	86.1
Implied Share Price (\$)	\$148.56	\$210.06	\$110.26
Upside / (Downside) %	(25%)	5%	(45%)

Sensitivity: Implied Upside / (Downside) - Exit Multiple x FY2028E EBITDA less NCI									
EBITDA ↓ / Multiple →	6.33x	6.58x	6.83x	7.08x	7.33x	7.58x	7.83x	8.08x	8.33x
2,550	(66%)	(62%)	(58%)	(55%)	(51%)	(47%)	(44%)	(40%)	(36%)
2,700	(60%)	(56%)	(52%)	(49%)	(45%)	(41%)	(37%)	(33%)	(29%)
2,850	(55%)	(51%)	(47%)	(42%)	(38%)	(34%)	(30%)	(26%)	(22%)
3,000	(49%)	(45%)	(41%)	(36%)	(32%)	(27%)	(23%)	(19%)	(14%)
3,150	(44%)	(39%)	(35%)	(30%)	(25%)	(21%)	(16%)	(12%)	(7%)
3,300	(38%)	(33%)	(29%)	(24%)	(19%)	(14%)	(9%)	(5%)	0%
3,450	(33%)	(28%)	(23%)	(18%)	(13%)	(8%)	(3%)	3%	8%
3,600	(27%)	(22%)	(17%)	(11%)	(6%)	(1%)	4%	10%	15%
3,750	(22%)	(16%)	(11%)	(5%)	0%	6%	11%	17%	22%

How could it go wrong?

Tenet insources anesthesia or signs a multi-year fixed-rate contract, and the 15.6% stipend growth reverts toward the 9.2% national rate, so the cost never arrives and the thesis dies. Mitigant: Tenet has not had a normal anesthesia year since 2015, its slowest was the 2020 shutdown at +4.7%, and Texas statute limits nurse-anesthetist (CRNA) scope in its largest state. A Q2 disclosure of a multi-year fixed-rate contract would damage the thesis, and I would cover.

Same-facility cases hit 1.6% in Q4 2025, and sustained growth above 3% with per-case revenue holding would outrun the cost. Mitigant: cases averaged 0.3% across the nine quarters to Q1 2026 and fell to -0.3% in Q1 2026, and the surgeons who drive that volume are retiring with no replacement visible in the royalty data.

Tenet retires partner stakes at 4.0x, removing their share permanently, and it has the free cash flow to do it. Mitigant: buyouts ran \$92M in FY2025 against \$809M of distributions, physicians hold appreciating stakes and rarely sell early, and the buyout retires the equity while the referral stream leaves with the surgeon.

USPI's per-case revenue growth of 5.6% in Q1 2026 could partly reflect the anesthesia stipend being recovered through higher commercial facility rates, the bull's case for why the cost never reaches margin. Mitigant: that growth slowed from 9.1% a year earlier while stipend inflation held near 15% a year, so any pass-through is losing ground to the cost rather than covering it.

Appendix

#1 Insider activity

Ten Tenet insiders sold \$38.9M of stock on the open market in the year till July 14 2026, while having no buyers. The sellers are the six most senior officers and a director: CEO Saumya Sutaria, COO Lisa Foo, CIO Paola Arbour, General Counsel Thomas Arnst, Principal Accounting Officer Scott Ramsey, and Tammy Romo. The \$38.9M counts only discretionary open-market sales, and excludes \$70.9M of shares withheld to pay taxes on vesting.

CEO pay actually paid was \$149.4M for 2025, well above the \$43.1M granted, because the equity awards were marked up as the stock ran. Management's awards swelled on the +151% move, and management is selling into it. Tenet also filed three management-turnover 8-Ks over the year, the most recent on April 10 2026.

#2 Reverse DCF

Reverse DCF: Implied Share Price (\$)						
Growth in Adj EBITDA less NCI/NCI %	16.00%	18.00%	20.00%	22.00%	24.00%	
0.0%	\$ 209	\$ 201	\$ 193	\$ 185	\$ 177	
-1.0%	\$ 195	\$ 188	\$ 180	\$ 173	\$ 165	
-2.0%	\$ 183	\$ 175	\$ 168	\$ 161	\$ 154	
-3.0%	\$ 171	\$ 164	\$ 157	\$ 151	\$ 144	
-4.0%	\$ 160	\$ 153	\$ 147	\$ 141	\$ 135	

A reverse DCF at Tenet's 11% cost of equity shows the \$199.20 price already assumes growth Tenet will not deliver, and it turns on EBITDA after the partners' cut, as a percentage of revenue.

Tenet guides partner distributions up 15.6% while EBITDA stays flat, and the anesthesia bill compounds at 15.6%, so the shareholder margin falls to 14.6% by FY2028 in my base case. At 14.6%, the same \$199.20 price requires revenue to grow 5.7% a year, nearly double consensus, in a year Tenet has already guided same-facility growth down to 3.0-6.0% from 7.5%. The stock is cheap only if that margin holds, and the margin is the whole short.

#3 Revenue & Operating Model

1. USPI (HCA) REVENUE BUILD									
(\$ in millions, unless otherwise noted)									
	FY2020	FY2021	FY2022	FY2023	FY2024	FY2025	FY2026	FY2027	FY2028
USPI Net Operating Revenue (RM)	\$ 1,860	\$ 4,334	\$ 5,171	\$ 5,870	\$ 6,346	\$ 6,812	\$ 7,286	\$ 7,680	\$ 8,085
YTD Growth %		17.1%	19.1%	13.6%	7.6%	7.2%	6.9%	5.4%	4.9%
USPI Negative Costs (RM)	\$ 960	\$ 1,077	\$ 1,148	\$ 1,207	\$ 1,264	\$ 1,321	\$ 1,381	\$ 1,443	\$ 1,507
YTD Growth %		10.1%	6.6%	4.7%	3.7%	4.2%	3.7%	3.7%	3.7%
Net Revenue per Case (\$)	\$ 4.026	\$ 4.289	\$ 4.505	\$ 4.726	\$ 4.980	\$ 5.190	\$ 5.355	\$ 5.555	\$ 5.755
YTD Growth %		6.5%	5.0%	4.7%	4.5%	4.3%	4.3%	4.3%	4.3%
2. HOSPITAL OPERATIONS (HOS) REVENUE BUILD									
ANNUAL									
	FY2020	FY2021	FY2022	FY2023	FY2024	FY2025	FY2026	FY2027	FY2028
SAME-STORE VOLUME									
Adjusted admissions same-store (actual)	962,962	1,022,117	1,042,920	1,062,823	1,082,726	1,102,629	1,122,532	1,142,435	1,162,338
YTD growth %		6.3%	2.0%	1.9%	1.9%	1.9%	1.9%	1.9%	1.9%
Rate									
Base				8.3%	8.2%	8.1%	8.0%	7.9%	7.8%
Base case				8.3%	8.2%	8.1%	8.0%	7.9%	7.8%
Selected Rate				8.3%	8.2%	8.1%	8.0%	7.9%	7.8%
Rate of Increase				8.3%	8.2%	8.1%	8.0%	7.9%	7.8%
REVENUE ADJUSTMENT									
Lengths of stay (per day)									
Per revenue per adjusted hospital (RM)	\$ 70	\$ 71	\$ 72	\$ 73	\$ 74	\$ 75	\$ 76	\$ 77	\$ 78
Revenue returned to year (RM) 90% mid-year	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -
Adjusted revenue (per day) (Revenue (RM))	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -	\$ -
TOTAL REVENUE SUMMARY									
USPI segment revenue (RM)	\$ 1,860	\$ 4,334	\$ 5,171	\$ 5,870	\$ 6,346	\$ 6,812	\$ 7,286	\$ 7,680	\$ 8,085
YTD growth %		17.1%	19.1%	13.6%	7.6%	7.2%	6.9%	5.4%	4.9%
Hospital Operations & Services segment revenue (RM)	\$ 16,880	\$ 16,181	\$ 16,180	\$ 16,181	\$ 16,182	\$ 16,183	\$ 16,184	\$ 16,185	\$ 16,186
YTD growth %		(4.3%)	(0.0%)	(0.0%)	(0.0%)	(0.0%)	(0.0%)	(0.0%)	(0.0%)
24 Same-store hospital NPIR-47 (RM)	\$ 16,880	\$ 16,181	\$ 16,180	\$ 16,181	\$ 16,182	\$ 16,183	\$ 16,184	\$ 16,185	\$ 16,186
28 State-Related Payments (RM)	\$ 829	\$ 1,343	\$ 1,338	\$ 1,333	\$ 1,328	\$ 1,323	\$ 1,318	\$ 1,313	\$ 1,308
NC Other Hospital revenue (Contract) (RM)	\$ 886	\$ 1,000	\$ 1,000	\$ 1,000	\$ 1,000	\$ 1,000	\$ 1,000	\$ 1,000	\$ 1,000
Total Tenet net operating revenue (RM)	\$ 20,546	\$ 20,865	\$ 21,510	\$ 21,882	\$ 21,862	\$ 21,115	\$ 22,482	\$ 22,482	\$ 22,482
YTD growth %		15.5%	3.1%	1.7%	(0.1%)	-3.4%	6.2%	0.0%	0.0%

Operating Model									
ANNUAL									
	FY2020	FY2021	FY2022	FY2023	FY2024	FY2025	FY2026	FY2027	FY2028
SALARIES, WAGES AND BENEFITS (DIRECT COST)									
Salaries & Wages and Benefits	\$ 4,500	\$ 4,600	\$ 4,700	\$ 4,800	\$ 4,900	\$ 5,000	\$ 5,100	\$ 5,200	\$ 5,300
YTD % Revenue	24.1%	24.5%	24.5%	24.5%	24.5%	24.5%	24.5%	24.5%	24.5%
Base case									
Base case									
Selected Rate									
Rate of Increase									
SUPPLY CHAIN COST									
Supplies	\$ 1,750	\$ 1,750	\$ 1,750	\$ 1,750	\$ 1,750	\$ 1,750	\$ 1,750	\$ 1,750	\$ 1,750
YTD % Revenue	9.4%	9.4%	9.4%	9.4%	9.4%	9.4%	9.4%	9.4%	9.4%
Base case									
Base case									
Selected Rate									
Rate of Increase									
GROSS MARGIN %									
Gross Margin %	39.1%	41.0%	42.7%	44.5%	46.3%	48.1%	49.9%	51.7%	53.5%
OTHER OPERATING EXPENSES - OVERHEAD									
Other Operating Expenses (Total)	\$ 4,500	\$ 4,600	\$ 4,700	\$ 4,800	\$ 4,900	\$ 5,000	\$ 5,100	\$ 5,200	\$ 5,300
YTD % Revenue	24.1%	24.5%	24.5%	24.5%	24.5%	24.5%	24.5%	24.5%	24.5%
Base case									
Base case									
Selected Rate									
Rate of Increase									
Adjusted EBITDA	\$ 3,800	\$ 3,900	\$ 4,000	\$ 4,100	\$ 4,200	\$ 4,300	\$ 4,400	\$ 4,500	\$ 4,600
Adj EBITDA Margin %	17.2%	19.1%	21.0%	22.9%	24.8%	26.7%	28.6%	30.5%	32.4%
YTD Adj EBITDA Growth %		11.6%	10.5%	10.5%	10.5%	10.5%	10.5%	10.5%	10.5%
Depreciation & Amortization	\$ 4,200	\$ 4,300	\$ 4,400	\$ 4,500	\$ 4,600	\$ 4,700	\$ 4,800	\$ 4,900	\$ 5,000
YTD % Revenue	20.4%	20.6%	20.4%	20.6%	20.6%	20.6%	20.6%	20.6%	20.6%
SG&A/Operating Income	\$ 4,200	\$ 4,300	\$ 4,400	\$ 4,500	\$ 4,600	\$ 4,700	\$ 4,800	\$ 4,900	\$ 5,000
YTD % Revenue	20.4%	20.6%	20.4%	20.6%	20.6%	20.6%	20.6%	20.6%	20.6%
INDEBTED, TAX & NCI									
Interest Expense	\$ 800	\$ 800	\$ 800	\$ 800	\$ 800	\$ 800	\$ 800	\$ 800	\$ 800
Other Non-Operating Income / Expense	\$ 100	\$ 100	\$ 100	\$ 100	\$ 100	\$ 100	\$ 100	\$ 100	\$ 100
Pre-tax Income (EATM)	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800
Effective Tax Rate %									
Net Income (EATM)	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800	\$ 1,800

I do not hold a position with the issuer such as employment, directorship, or consultancy.

I and/or others I advise do not hold a material investment in the issuer's securities.

Catalyst

July 24, 2026, Q2 2026 earnings. HCA pre-announced its \$400M payer-mix hit ten days before its own print, and Tenet has not. Watch Ambulatory same-facility cases against the 0.3% run rate, per-case revenue against the 5.6% Q1 print, and NCI against the \$865-895M Ambulatory guide.

Q3 2026 through Q2 2027, anesthesia contract renewals. Each print narrows the gap between the reported segment margin and the cost the HCRIS filings show.

December 31, 2026, Conifer stops serving CommonSpirit. The exit removes about \$630M of recurring revenue from FY2027. Tenet receives a \$1.9B termination payment, roughly \$1.65B of it in 2026, so 2026 cash is flattered while the recurring earnings leave in FY2027.

2027-2028, Medicaid supplemental cuts begin against a \$1,338M base, 8% of hospital revenue. The 2005-2007 NPI registrants begin retiring at the 39 facilities that hold the royalty concentration.

Between prints. New CMS HCRIS filings for the eleven USPI surgical hospitals, filed annually, are the cleanest read on stipend escalation.

Messages

Subject	HCA vs THC
Entry	07/24/2026 09:26 AM
Member	riskintolerance

Anyone have a read on how the same factors (exchange roll-off, higher supplemental payments, lower surgeries, etc.) impacted THC so positively and HCA so negatively in the quarter? Quite a wide spread for two companies in the same industry and operating environment.

Subject	I think you mean SGRY, not THC
Entry	07/24/2026 11:03 AM
Member	ElmSt14

If you want to focus on a shady hospital company with significant leverage, questionable business/billing practices, deceptive accounting and excessive valuation, I would point you to SGRY and not THC.

SGRY is the poster child for many of the points you bring up and if you simply look at FCF defined either as EBITDA - capex - interest - taxes - cash distributions to NCI or CFFO - capex - NCI, you can clearly see the difference (without even getting into management credibility and capital allocation skills):

SURGERY PARTNERS, INC.
CONSOLIDATED STATEMENTS OF CASH FLOWS
(Unaudited, dollars in millions)

	Year Ended December 31,		
	2025	2024	2023
Cash flows from operating activities:			
Net income	\$ 98.9	\$ 12.5	\$ 135.3
Adjustments to reconcile net income to net cash provided by operating activities:			
Depreciation and amortization	176.0	152.6	118.1
Non-cash lease expense	37.8	38.9	35.2
Non-cash interest expense, net	9.6	6.5	25.0
Equity-based compensation expense	14.8	33.3	17.7
Net loss on disposals, consolidations and deconsolidations	30.4	40.6	14.4
Loss on debt extinguishment	1.3	5.1	15.5
Deferred income taxes	16.7	131.5	(1.7)
Equity in earnings of unconsolidated affiliates, net of distributions received	(0.8)	(2.0)	(2.2)
Changes in operating assets and liabilities, net of acquisitions and divestitures:			
Accounts receivable	(53.8)	(72.1)	(47.2)
Other operating assets and liabilities	(56.6)	(46.8)	(16.3)
Net cash provided by operating activities	274.3	300.1	293.8
Cash flows from investing activities:			
Purchases of property and equipment	(78.7)	(90.4)	(88.8)
Payments for acquisitions, net of cash acquired	(162.1)	(378.8)	(80.0)
Proceeds from disposals of facilities and other assets	46.5	2.6	25.8
Purchases of equity investments	(13.3)	(1.7)	(50.3)
Proceeds from sales of equity investments	1.5	4.6	1.4
Other investing activities	(40.5)	(24.8)	(33.7)
Net cash used in investing activities	(246.6)	(488.5)	(225.6)
Cash flows from financing activities:			
Principal payments on long-term debt	(1,039.8)	(1,279.3)	(807.1)
Borrowings of long-term debt	1,202.6	1,729.7	826.6
Payments of debt issuance costs	(4.1)	(14.9)	(24.3)
Distributions to non-controlling interest holders	(226.0)	(170.5)	(146.1)
Proceeds (payments) related to ownership transactions with non-controlling interest holders	0.6	9.6	8.2
Other financing activities	9.4	(12.6)	(12.5)
Net cash (used in) provided by financing activities	(57.3)	262.0	(155.2)
Net (decrease) increase in cash and cash equivalents	(29.6)	73.6	(87.0)
Cash and cash equivalents at beginning of period	269.5	195.9	282.9
Cash and cash equivalents at end of period	\$ 239.9	\$ 269.5	\$ 195.9
Supplemental cash flow information:			
Interest paid, net of interest income received	256.8	199.7	169.6
Cash paid for income taxes	1.2	1.6	1.4
Non-cash purchases of property and equipment	14.2	12.9	18.0

TENET HEALTHCARE CORPORATION
CONSOLIDATED STATEMENTS OF CASH FLOWS
(Unaudited)

	Years Ended December 31,	
	2025	2024
<i>(Dollars in millions)</i>		
Net income	\$ 2,367	\$ 4,064
Adjustments to reconcile net income to net cash provided by operating activities:		
Depreciation and amortization	863	818
Deferred income tax expense (benefit)	9	(103)
Stock-based compensation expense	104	67
Impairment and restructuring charges, and acquisition-related costs	130	102
Litigation and investigation costs	64	35
Net losses (gains) on sales, consolidation and deconsolidation of facilities	1	(2,916)
Loss from early extinguishment of debt	4	8
Equity in earnings of unconsolidated affiliates, net of distributions received	(34)	(29)
Amortization of debt discount and debt issuance costs	23	26
Net gains from the sale of investments and long-lived assets	(4)	(4)
Other items, net	(6)	(4)
Changes in cash from operating assets and liabilities:		
Accounts receivable	20	245
Inventories and other current assets	(73)	(86)
Income taxes	(25)	16
Accounts payable, accrued expenses and other current liabilities	209	(30)
Other long-term liabilities	9	(9)
Payments for restructuring charges, acquisition-related costs, and litigation costs and settlements	(121)	(153)
Net cash provided by operating activities	3,540	2,047
Cash flows from investing activities:		
Purchases of property and equipment	(4,010)	(931)
Purchases of businesses or joint venture interests, net of cash acquired	(308)	(571)
Proceeds from sales of facilities and other assets	38	4,981
Proceeds from sales of marketable securities and long-term investments	93	63
Purchases of marketable securities and long-term investments	(90)	(94)
Other items, net	2	(19)
Net cash provided by (used in) investing activities	(1,275)	3,429
Cash flows from financing activities:		
Repayments of borrowings	(2,372)	(2,243)
Proceeds from borrowings	2,276	23
Repurchases of common stock	(1,386)	(672)
Debt issuance costs	(32)	—
Distributions paid to noncontrolling interests	(609)	(681)
Proceeds from the sale of noncontrolling interests	42	23

Subject
Entry
Member

Re: HCA vs THC
07/24/2026 03:30 PM
levisherr

Hi, thank you so much for reading through the pitch and asking the question.

As per my research, HCA and THC, despite both having a portfolio of hospitals, is due to different exposure of exchange enrollments (marketplace insurances) by the end of Obamacare's extra government subsidy that initially made these marketplace insurances cheaper. Here is a more detailed overview:

#1 Exposure of Enrollment Patients

Tenet estimates the exposure to around 6.5% of total revenue from 7.5% admissions volume, while HCA in their 2025's management disclosure stated it to be around 8% of volume and 10% of revenue.

	Florida	Texas	FL + TX	Total	FL+TX share
Marketplace enrollees, 2025 (CMS OEP Report)	4,735,415	3,966,226	8,701,641	24,166,491	36% of US
HCA hospitals (HCA 10-K, Dec 2025)	47	55	102	190	54%
HCA licensed beds	13,384	14,595	27,979	50,436	55%
Tenet hospitals (Tenet 10-K, Dec 2025)	5	15	20	49	41%

HCA's hospitals are also more concentrated relative to THC (55% vs 41%) in Florida and Texas, having about 36% of national marketplace enrollees. Further, HCA's own admissions from these patients had risen 43-45% in 2024 which has led to this \$1.0 to \$1.2 billion profit hit estimate for the year.

#2 Primary Business Segment Mix

Due to a law called EMTALA, any hospital with an emergency room must screen and treat anyone with an emergency, regardless of insurance or ability to pay (as per CMS and ACEP). Contrary to this, an ambulatory surgery center (which makes up roughly 0-45% of THC's profits) does not have emergency room and consequently no such duties, so it verifies insurance and collects payment before it schedules a case.

Recent coverage losses show this directly, with uninsured emergency-department visits rising as coverage falls with hospitals' uncompensated-care rate climbing from 6.4% to 8.7% of business through 2023. This is HCA's core business, making up 62% of its revenue under the segment named "inpatient care". As Tenet is moving towards ASCs, which by nature only do planned operations for insured people, it is relatively insulated from this risk of emergency visits by uninsured.

#3 Medicare Payments Offsetting the Expenses

Both companies offset the exchange damage with Medicaid state-directed payments, which are extra dollars states route to hospitals through their managed-care plans on top of the normal rate in large amounts. Approved arrangements nationally were projected at \$110.2 billion a year as of August 2024.

Tenet's share has grown steadily, from \$929M in 2023 to \$1,338M in 2025, equal to 9.6% of its hospital revenue, which easily covers its small \$250M exchange hit. HCA leans on the same programs, helped this quarter by a newly approved Florida directed payment program, but its \$1.0 to \$1.2 billion exchange hit is far too large for them to fully offset, which is why HCA still cut its forecast.

Importantly, the 2025 One Big Beautiful Bill Act, caps these payments at 110% of Medicare rates in non-expansion states like Florida and Texas, and phases any amount above that cap down by 10 percentage points a year starting January 1, 2028 (OBBBA Section 71116). It also cuts the provider taxes that fund them, from about 6% to 3.5% by 2032 (Commonwealth Fund, 2025).

This does not nullify a short thesis on THC, however, the timeline and the catalyst timings, now also include 1 Jan 2028 along with the existing catalysts. The competitive threats of UHC in the surgery center segment of THC, along with anesthesia cost pressures, and slowing down of a business mix shift is still pertinent which I believe shall drag down the company's profitability in the long-term.